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WHITE PAPER

CMS TEAM in New York: 78 Mandated Hospitals, the Nation's Largest Bundled-Payment Market

Live roster counts and CMS readmissions data

Jennifer Tim-Diamond for Rainfall Health

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01 · At a glance

Live hospital counts come from `data/territoryHospitals.json` (March 2026 CMS TEAM participant extract). The HRRP row below is computed at render time from the CMS HRRP dataset (9n3s-kdb3) joined to this scope's roster by CCN; Medicare–population and per–episode LEJR figures are not published at hospital level, so this report states that rather than estimating them.

Table 1

Signal	Value	Why it matters
Mandated hospitals	78	Largest single–state TEAM market
CBSAs	6	Largest CBSA: New York–Newark–Jersey City, NY–NJ (65 hospitals)
Medicare beneficiaries	Not computed in this pipeline	Covered–lives scale
HRRP (all conditions)	48.7% vs 48.1% national	Readmission pressure under episode accountability
Est. annual LEJR	Not published at hospital level	Volume concentration

02 · Where the mandate sits

Largest market in this scope: **New York–Newark–Jersey City, NY–NJ** (65 hospitals).



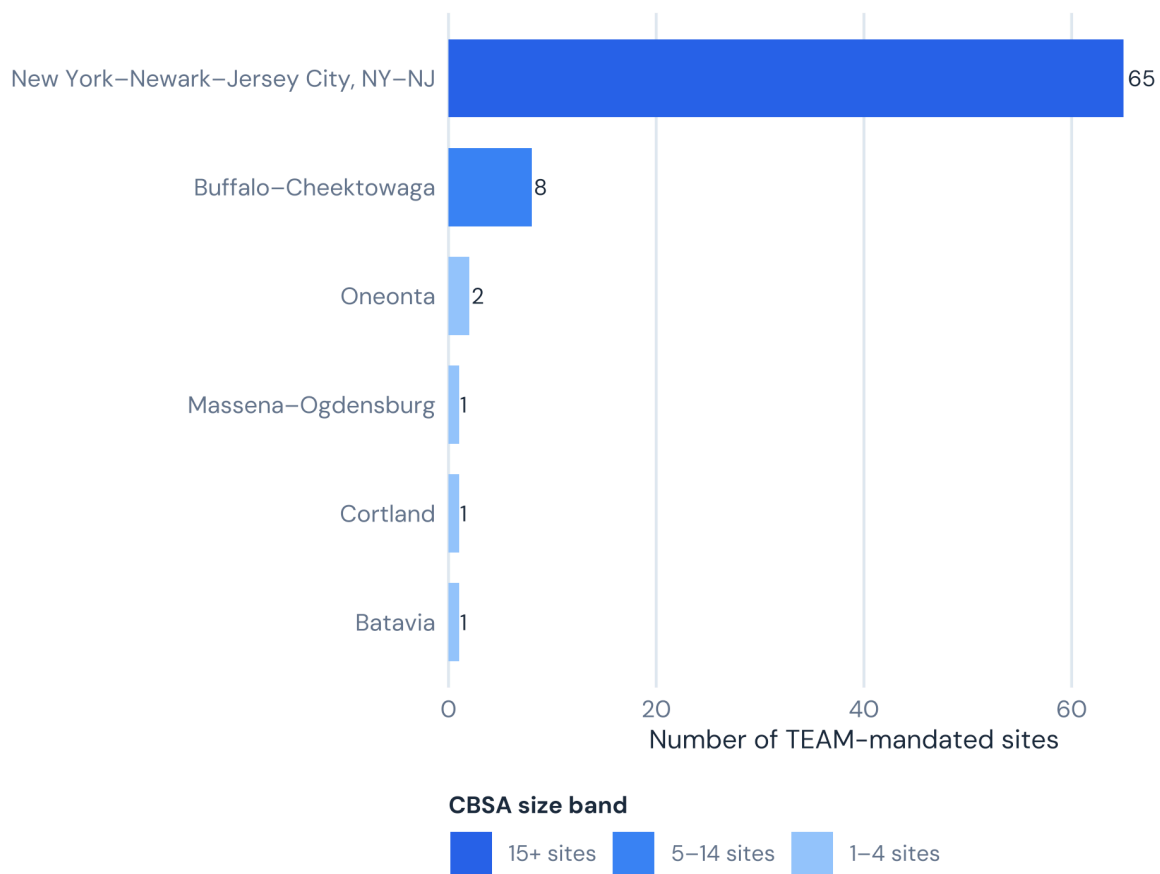


Figure 1: CMS TEAM-mandated hospitals in this scope.

Table 2

CBSA	Hospitals	Share (%)
New York–Newark–Jersey City, NY–NJ	65	83.3
Buffalo–Cheektowaga	8	10.3
Oneonta	2	2.6
Batavia	1	1.3
Cortland	1	1.3
Massena–Ogdensburg	1	1.3

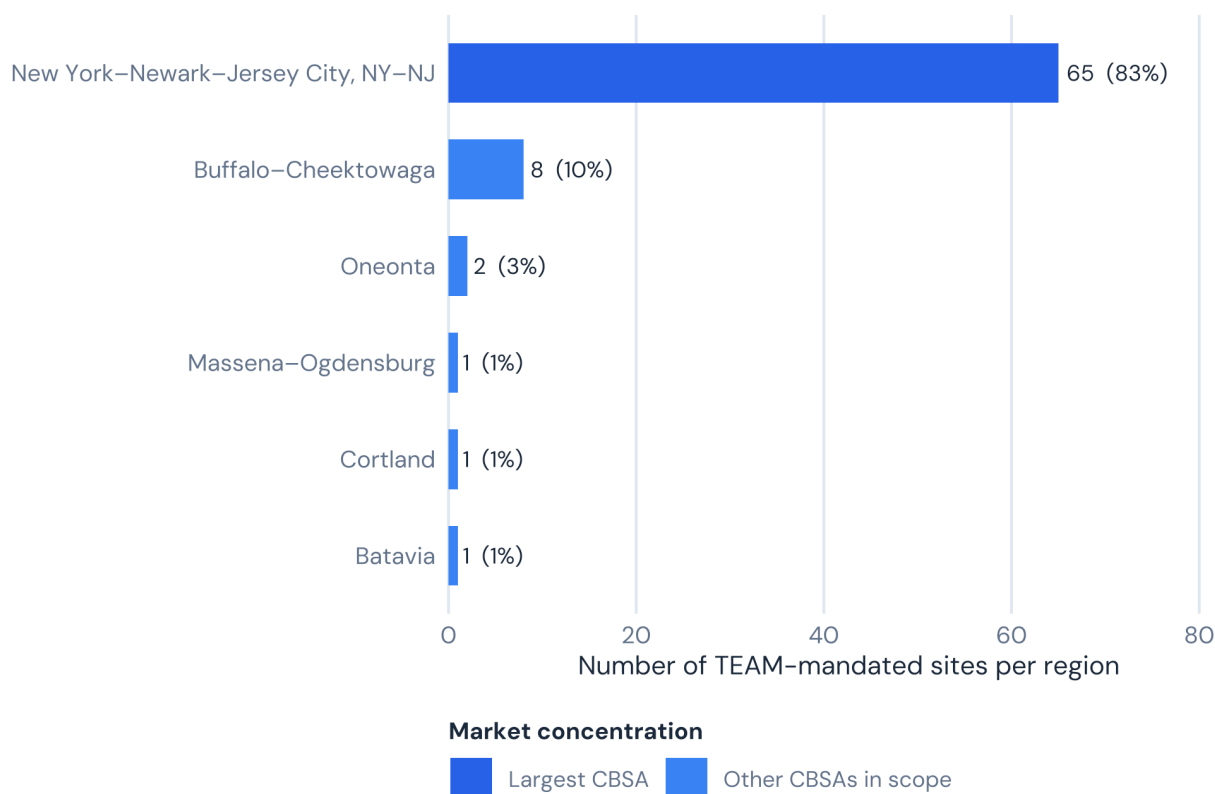


Figure 2: Concentration of TEAM-mandated hospitals.

03 • Guide narrative (from the website TEAM report)

Takeaways

- 78 hospitals mandated — second nationally
- 65 in New York–Newark–Jersey City metro
- New York’s HRRP excess-readmission rate is essentially at the national baseline, not meaningfully worse
- NYU Langone, Mount Sinai, Montefiore in scope
- Hip/knee replacement is TEAM’s core procedure category, and New York’s hip/knee readmission rate runs below the national rate

New York and the TEAM mandate: market scope and stakes

New York State has **78 hospitals** mandated to participate in the CMS Transforming Episode Accountability Model across **six Core-Based Statistical Areas**, making it the second-largest TEAM state in the country behind California. TEAM covers five episode categories — lower extremity joint replacement (LEJR), surgical hip/femur fracture treatment, spinal fusion, major bowel procedures, and coronary artery bypass graft — but CMS does not publish hospital-level episode volumes, so this report does not model a New York-specific episode count.

The geographic concentration is striking. Sixty-five of the 78 mandated hospitals — **83% of the state total** — are located within the New York–Newark–Jersey City, NY–NJ metropolitan CBSA, the nation’s largest metropolitan bundled-payment market. This multi-state CBSA also contains all 44 mandated New Jersey hospitals, creating a shared post-acute ecosystem spanning two states, five boroughs, and suburban markets from Long Island to Westchester to northern New Jersey.

New York’s hospital market is among the most structurally complex in the country. Large nonprofit health systems (Northwell Health, Mount Sinai Health System, NYU Langone, NewYork–Presbyterian, Montefiore)

coexist with city-operated public hospitals (NYC Health + Hospitals), academic medical centers, and smaller community facilities. This heterogeneity shapes TEAM performance unevenly: across the CMS HRRP dataset, New York's TEAM-mandated hospitals as a group post a hip/knee excess-readmission rate below the national average, but individual large systems still show pockets of elevated risk — Mount Sinai West and Mount Sinai South Nassau both post hip/knee ERRs above 1.25, among the highest in the state's roster.

(Source: Centers for Medicare & Medicaid Services (2026); Rainfall Health analysis.)

Fast facts

- 78 hospitals mandated (March 2026 CMS list)
- 6 CBSAs: New York–Newark–Jersey City (65), Buffalo–Cheektowaga (8), Oneonta (2), Batavia (1), Cortland (1), Massena–Ogdensburg (1)
- CMS does not publish hospital-level LEJR episode volumes; this report does not estimate one for New York
- 48.7% of NY hospital-condition pairs in the CMS HRRP file exceed ERR 1.0 — essentially at the 48.1% national rate, not meaningfully worse
- New York's hip/knee-replacement ERR is below the national rate: 24.4% of NY hip/knee pairs exceed 1.0, versus 47.9% nationally
- Top health systems by roster presence: Northwell Health, Mount Sinai, NYU Langone, Montefiore, NewYork–Presbyterian, NYC Health + Hospitals

Market context

The state's hospital market is dominated by large nonprofit systems that depend on Medicare as a primary revenue source, making TEAM's financial implications consequential for the sector. This repo does not carry a verified per-state Medicare beneficiary count, so that figure is omitted rather than estimated.

Hospital costs in the New York metropolitan area are widely reported to run above national benchmarks, driven by labor costs, real estate expenses, and the density of academic and specialty care. CMS sets TEAM target prices using each hospital's own historical spending blended with regional benchmarks, so a hospital's own cost structure is partially reflected in its target; the 2.0% discount factor CMS applies to LEJR, SHFFT, and spinal fusion targets still creates a gap that must be closed through care coordination, not just baseline cost levels.

A landmark JAMA study by Dummit et al. (2016) analyzing the predecessor Comprehensive Joint Replacement (CJR) model — which also included New York markets — found that participating hospitals reduced episode payments by an average of \$864 per episode in the first performance year. Finkelstein et al. (2021), analyzing mandatory bundled payments in lower extremity joint replacement, found that discharge to institutional post-acute care declined significantly under mandatory programs. Neither study isolates a New York-specific effect; they are cited here as the relevant evidence base for what mandatory bundling has done elsewhere, not as New York figures.

(Source: Dummit et al. (2016), JAMA 316(12).) (Source: Finkelstein et al. (2021), JAMA 325(18).)

Complete hospital directory by CBSA

The following table lists every CMS TEAM-mandated hospital in New York, organized by Core-Based Statistical Area, generated at render time from the same live roster used for the counts above — so it cannot drift out of sync with them or omit a participant.

Table 3

Hospitals in this CBSA

65	New York	Bellevue Hospital Center · Bronxcare Hospital Center · Brookdale Hospital Medical Center · Brooklyn Hospital Center – Downtown Campus · Elmhurst Hospital Center · Flushing Hospital Medical Center · Good Samaritan Hospital · Good Samaritan Hospital of Suffern · Harlem Hospital Center · Helen Hayes Hospital · Hospital for Special Surgery · Hudson Valley Hospital Center · Huntington Hospital · Jacobi Medical Center · Jamaica Hospital Medical Center · John t Mather Memorial Hospital of Port Jefferson · Kings County Hospital Center · Lenox Hill Hospital · Lincoln Medical & Mental Health Center · Long Island Community Hospital · Long Island Jewish Medical Center · Maimonides Medical Center · Mercy Medical Center · Metropolitan Hospital Center · Montefiore Medical Center · Montefiore Mount Vernon Hospital · Montefiore New Rochelle Hospital · Mount Sinai Beth Israel · Mount Sinai Hospital · Mount Sinai South Nassau · Mount Sinai West · New York Eye and Ear Infirmary · Nassau University Medical Center · New York Community Hospital of Brooklyn, Inc. · New York–Presbyterian Hospital · New York–Presbyterian/Queens · North Shore University Hospital · Northern Westchester Hospital · Nyack Hospital · Nyu Langone Hospitals · Peconic Bay Medical Center · Phelps Hospital · Plainview Hospital · Putnam Hospital Center · Queens Hospital Center · Richmond University Medical Center · South Brooklyn Health · South Shore University Hospital · St Barnabas Hospital · St Catherine of Siena Hospital · St Charles Hospital · St Francis Hospital, Roslyn · St John’s Episcopal Hospital at South Shore · St John’s Riverside Hospital · St Joseph Hospital · St Joseph’s Medical Center · Staten Island University Hospital · Stony Brook University Hospital · Suny Health Science Center at Brooklyn University · Terence Cardinal Cooke Health Care Center · Westchester Medical Center · White Plains Hospital Center · Woodhull Medical & Mental Health Center · Wyckoff Heights Medical Center
8	Buffalo	Bertrand Chaffee Hospital · Erie County Medical Center · Kaleida Health · Kenmore Mercy Hospital · Mercy Hospital of Buffalo · Mount St Mary’s Hospital and Health Center · Niagara Falls Memorial Medical Center · Sisters of Charity Hospital
2	Albany	Osborn Fox Memorial Hospital · Mary Imogene Bassett Hospital
1	Baltimore	United Memorial Medical Center
1	Butte	Cuthrie Cortland Regional Medical Center
1	Massachusetts	Potsdam Hospital

(Source: Centers for Medicare & Medicaid Services (2026). TEAM participant list.)

The New York–Newark–Jersey City CBSA: a shared post-acute market

The New York–Newark–Jersey City, NY–NJ CBSA is not simply a large metropolitan area — it is a geographically integrated care delivery system in which a patient discharged from a Bronx hospital may recover at a skilled nursing facility in New Jersey, and vice versa. This market integration creates both competitive complexity and coordinating opportunity for TEAM management.

The CBSA contains some of the country’s most well-resourced hospitals. **Hospital for Special Surgery (HSS)** — internationally recognized as the top-ranked orthopedic hospital in the United States — is a TEAM



participant, as are NYU Langone (ranked among the top hospitals nationally for orthopedics by U.S. News & World Report), North Shore University Hospital, Lenox Hill Hospital, and Stony Brook University Hospital. These institutions' historical episode efficiency will influence their target prices; facilities with strong prior CJR performance enter TEAM with tighter targets but also with post-acute coordination infrastructure already in place.

The presence of the NYC Health + Hospitals system — including Bellevue, Kings County, Lincoln Medical, Queens Hospital, Harlem Hospital, Elmhurst, Jacobi, Woodhull, and Metropolitan — introduces a distinct policy dimension. These public hospitals disproportionately serve uninsured and dual-eligible patients, whose post-discharge resources and social determinants of health differ substantially from the commercial patient mix at neighboring academic centers. Under TEAM, risk adjustment partially accounts for patient complexity, but research has consistently found that safety-net hospitals face structural TEAM disadvantages (Navathe et al., 2020).

The large Northwell Health system — which operates North Shore University Hospital, Long Island Jewish Medical Center, South Shore University Hospital, Lenox Hill, Huntington Hospital, Peconic Bay Medical Center, and others — gives it an unusual degree of coordinated leverage under TEAM. Northwell's ability to direct patients to its own post-acute partners and monitor episode costs across a vertically integrated network is a structural advantage that independent community hospitals in the same CBSA cannot match.

(Source: Navathe et al. (2020), JAMA 323(2).)

Buffalo and upstate CBSAs

Western New York's **Buffalo–Cheektowaga CBSA** adds eight hospitals, anchored by **Kaleida Health** — the region's dominant health system operating Buffalo General Medical Center, Women & Children's Hospital, Gates Vascular Institute, and affiliated facilities — and the **Erie County Medical Center**, the county's Level 1 trauma center and safety-net hospital. Sisters of Charity Hospital and Mercy Hospital of Buffalo represent the Catholic Health system presence. The smaller community hospitals (Kenmore Mercy, Mount St. Mary's, Niagara Falls Memorial, Bertrand Chaffee) serve suburban and rural catchment areas with different post-acute landscapes than the urban core.

The three upstate CBSAs — **Oneonta** (Mary Imogene Bassett Hospital and Aurelia Osborn Fox Memorial), **Batavia** (United Memorial Medical Center), and **Cortland** (Guthrie Cortland Regional) — along with **Canton–Potsdam Hospital** in the Massena–Ogdensburg market, represent critical access and rural community hospitals with constrained post-acute networks. For these facilities, TEAM's 30-day episode window is particularly difficult to manage: skilled nursing facility options are geographically limited, home health capacity is strained, and patients frequently travel significant distances from their county of residence for surgical care, complicating post-discharge follow-up.

Readmissions landscape: New York under HRRP

Joining the live CMS TEAM roster to the CMS HRRP dataset (9n3s-kdb3, performance period 07/01/2021–06/30/2024) by CCN, 75 of New York's 78 mandated hospitals appear in the HRRP file. Across those hospitals' condition pairs, **48.7%** exceed the excess readmission ratio (ERR) threshold of 1.0 — meaning more readmissions than CMS's risk model predicts. That is essentially in line with the **48.1%** national rate across the same dataset, not the outsized gap sometimes attributed to New York.

The picture varies sharply by condition. New York's **hip/knee replacement** ERR — the procedure category TEAM weights most heavily — is notably better than the national pattern: 24.4% of NY hip/knee pairs exceed 1.0, versus 47.9% nationally. New York also runs below the national rate on CABG (31.6% vs 49.9%). The state runs above the national rate on the medical conditions HRRP also tracks — heart attack, heart failure, pneumonia, and COPD — all of which sit outside TEAM's five bundled-payment categories. In short: on the readmission measure most relevant to TEAM specifically, New York's mandated hospitals outperform the

national pool; the state's readmissions profile looks worse only when medical (non-surgical) conditions are folded into a single blended rate.

Under CMS TEAM, readmissions still carry compounded consequences where they do occur: a patient readmitted within 30 days of a TEAM-qualifying procedure adds both an inpatient cost to the episode total (raising the risk of exceeding the target price) and a separate HRRP penalty event. Managing the first 30 days post-discharge remains the primary lever for TEAM financial performance even where a hospital's baseline readmission rate is favorable.

(Source: Centers for Medicare & Medicaid Services. HRRP supplemental data, Dataset 9n3s-kdb3. — vendored copy joined to the CMS TEAM participant list by CCN for this report.)

Financial exposure: what this report does and doesn't estimate

CMS does not publish hospital-level TEAM episode volumes or target prices in a form this repo can join to the New York roster, so this report does not model a New York-specific episode count, target price, or gain/loss dollar figure — doing so would mean presenting an invented number as if it were New York data. Two external findings are relevant as general context, not as New York-specific estimates:

- Optum Advisory (2025) estimated that 56% of TEAM-mandated hospitals nationally are projected to lose money once downside risk begins in Performance Year 2, with average losses of \$1.2 million per facility. This is a national estimate; Optum's methodology and hospital-level detail are not published in a form this report can verify or apply to New York specifically.
- HFMA (2026) recommends hospitals use Performance Year 1 (2026) — the upside-only year — to establish baseline episode cost data, identify high-cost post-acute partners, and build PCP follow-up protocols before two-sided risk begins in 2027.

A hospital that wants a New York-specific dollar estimate needs its own CMS-issued target price file, obtained through the TEAM participant portal — not a state-level average.

(Source: Optum Advisory (2025); Healthcare Financial Management Association (2026).)

What New York hospitals must prioritize in 2026

1. **Establish real-time episode cost baselines.** Hospitals must obtain their CMS-issued target price files through the TEAM participant portal and model their current trajectory against those targets before the PY1 reconciliation window closes.
2. **Build or formalize post-acute care networks.** The NYC metro area has hundreds of SNF and home health providers, but their quality and cost profiles vary dramatically. Hospitals should identify preferred post-acute partners whose episode costs align with TEAM targets — a process requiring structured data sharing agreements that take months to negotiate.
3. **Address the 7-day PCP follow-up requirement.** TEAM requires documentation of primary care follow-up within seven days of discharge for qualifying episodes. In New York City, where primary care access is fragmented and PCP shortage areas are common, operationalizing this requirement is a significant workflow challenge for social work and discharge planning teams.
4. **Differentiate Track 2 eligibility.** Safety-net hospitals in the NYC public hospital system — Bellevue, Kings County, Lincoln, Elmhurst, Queens — may qualify for Track 2 (5%/5% stop-gain/stop-loss) given their safety-net designations. Confirming eligibility and electing Track 2 before PY2 may reduce financial volatility for these facilities.
5. **Integrate HRRP and TEAM quality improvement.** New York's mandated hospitals already run below the national readmission rate on hip/knee replacement specifically, but above it on the medical conditions HRRP tracks. Reducing 30-day readmissions at TEAM procedure types reduces both HRRP penalties and episode costs at once — these programs should be integrated, not operated in parallel — and hospitals should not assume TEAM-specific risk is elevated just because their overall HRRP profile is.

Frequently asked questions

How many hospitals are mandated for CMS TEAM in New York?

78 hospitals in New York State are mandated to participate in CMS TEAM, based on the March 2026 CMS participant list.

Which CBSAs in New York are included in CMS TEAM?

Six CBSAs: New York–Newark–Jersey City, NY–NJ (65 hospitals); Buffalo–Cheektowaga, NY (8); Oneonta, NY (2); Batavia, NY (1); Cortland, NY (1); and Massena–Ogdensburg, NY (1).

Is Hospital for Special Surgery required to participate in CMS TEAM?

Yes. Hospital for Special Surgery (HSS) is listed on the CMS TEAM participant roster in the New York–Newark–Jersey City CBSA.

Is the entire Northwell Health system mandated?

Several Northwell Health hospitals are on the TEAM participant list, including North Shore University Hospital, Long Island Jewish Medical Center, South Shore University Hospital, Lenox Hill Hospital, Huntington Hospital, and Peconic Bay Medical Center, among others.

What is New York's HRRP excess-readmission rate?

48.7% of New York hospital–condition pairs exceed the ERR 1.0 threshold in the CMS HRRP dataset, versus 48.1% nationally — essentially at parity, not meaningfully worse. On hip/knee replacement specifically, the procedure category TEAM weights most heavily, New York runs below the national rate (24.4% vs 47.9%). HRRP's "excess readmission ratio" is a risk-adjustment measure, not a payment penalty rate on its own.

Are NYC public hospitals (Health + Hospitals) included in TEAM?

Yes. Multiple NYC Health + Hospitals facilities are on the TEAM roster, including Bellevue Hospital Center, Kings County Hospital Center, Lincoln Medical Center, Harlem Hospital Center, Elmhurst Hospital Center, Queens Hospital Center, Jacobi Medical Center, and Woodhull Medical Center.

When does downside financial risk begin for New York TEAM hospitals?

Performance Year 1 (2026) has no downside risk. Most hospitals transition to Track 3 (20%/20% stop–gain/stop–loss) starting in Performance Year 2 (January 1, 2027).

Does CMS TEAM apply to upstate New York hospitals?

Yes, but more selectively. CMS chose CBSAs based on regional surgical episode volume. The Buffalo area has 8 mandated hospitals; smaller upstate CBSAs (Oneonta, Batavia, Cortland, Massena–Ogdensburg) each have 1–2 hospitals. The vast majority of the state's TEAM exposure is concentrated in the NYC metro.

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